

Media Information

09-05-2025 18:35

Datavant

Fax Id:

RUCKER, Derrica (id)

3D DIAGNOSTIC BILATERAL MAMMOGRAM TOMO (#514114922, 08/21/2025 12:00am)

2025-08-21 16:5302 0000000000 >> 12P 3/4

Jersey

Diagnostic

Imaging

929 North Wood Ave. Linden NJ 07036 (P) 908-241-5322 (F) 908-241-0332
DIGITAL X-RAY • 3D MAMMOGRAPHY • ULTRASOUND • VASCULAR & ECHO ULTRASOUND • EKG

PATIENT'S NAME:

DATE OF BIRTH: 13-Oct-1991

DATE OF SERVICE: 20-Aug-2025 03:49:00 PM,20-Aug-2025 05:19:07 PM

PATIENT:

REF. DOCTOR:

Diagnostic Bilateral Mammogram with Tomosynthesis and Bilateral Ultrasound.

INDICATION: Left breast lump.

TECHNICAL FACTORS: Bilateral CC and MLO views were performed with tomosynthesis. Spot compression views of the left breast were done. Bilateral global sonography was performed.

COMPARISON:None Available.

BREAST DENSITY: B Scattered fibroglandular tissue is seen.

FINDINGS: A triangular skin marker has been placed on the upper central left breast, denoting the site of reported palpable abnormality. There is an underlying 4 cm mass with spiculated and indistinct margins. No
Bilateral complete breast ultrasound was done. There is a hypochoic mass with angular margins measuring 4.1 cm at the 11 o'clock position, 10 cm from the nipple in the left breast, corresponding to the area of reported palpable abnormality. An enlarged abnormal lymph node measures 3.1 cm in the left axilla. There is also an abnormal round lymph node measuring 1 cm in the left axilla as well.

CONCLUSION: Highly suspicious left breast mass at the 11 o'clock position with suspicious corresponding axillary lymph nodes. Surgical consultation and tissue sampling are advised. The findings and recommendation were discussed with Nurse Reed over the phone on August 21, 2025 at 1:00 p.m.

BIRADS CATEGORY: 5

BI-RADS CATEGORY AS PER ACR:

Category 0: Incomplete- Needs Additional Imaging Evaluation.

Category 1: Negative.

Category 2: Benign Finding - Routine Follow Up.

Category 3: Probably Benign Finding - Short Interval Follow up in 6 Months.

Category 4 (A/B/C): Suspicious Finding - Biopsy should be considered.

Category 5: Highly Suggestive of Malignancy - Appropriate Action Should be Taken Category 6: Biopsy Proven Malignancy

RUCKER, Derrica (id
2025-08-21 16:5402 >> 18557745474 12P 4/4



325 North Wood Ave. Linden NJ 07036 (P) 908-341-0322 (F) 908-341-0332
DIGITAL X-RAY • 3D MAMMOGRAPHY • ULTRASOUND • VASCULAR & ECHO ULTRASOUND • BRG

PATIENT'S NAME:
DATE OF BIRTH:
DATE OF SERVICE: 20-Aug-2025 09:49:00 PM,20-Aug-2025 05:19:07 PM
PATIENT ID#: RUCDE0001
REF. DOCTOR:

The false negative rate for mammography is 10-15%, and almost 50% for dense breasts. Palpable abnormalities must be managed clinically.
A letter summarizing these findings will be provided to the patient. As required by New York law the patient will be notified that she has dense breasts. Dense breast tissue can make it harder to find cancer on a mammogram and may also be associated with an increased risk of breast cancer. Performing breast ultrasound with mammography will detect more breast cancers than performing mammography alone, and bilateral breast ultrasound should be considered after discussion between the patient and clinician.

As per the FDA requirements, a layman's letter has been generated and sent to your printer stating the results and recommendations of this breast imaging study. The state of New Jersey has passed a law, May 1, 2014. This New Jersey breast density law requires all patients and their care providers, regardless of breast density, to receive the information below. Your mammogram now shows that you have dense breast tissue as determined by the breast imaging reporting and data system established by the American college of radiology. Dense breast tissue is very common and is not abnormal. However, in some cases, dense breast tissue can make it harder to find cancer on a mammogram and may also be associated with a risk factor for breast cancer. Discuss this and other facts for breast cancer that pertain to your personal medical history with your health care provider. A copy of your results were sent to your health care provider. You may also find more information about breast density at the website of the American college of radiology, www.acr.org.

Electronically Signed by BAI, VANONIA DR. at 21-Aug-2025 4:32 PM

RUCKER, Derrica (id [REDACTED])
3D DIAGNOSTIC BILATERAL MAMMOGRAM TOMO (#517273504, 08/20/2025 12:00am)
2025-09-04 10:28 02 0000000000 >> 18557745474 2 P 1/3



DIGITAL X-RAY • 3D MAMMOGRAPHY • ULTRASOUND • VASCULAR & ECHO ULTRASOUND • EKG

929 NORTH WOOD AVENUE • LINDEN, NJ 07036 • TEL: 908-241-5222 • FAX: [REDACTED]

August 21, 2025

Dear Dr. Shannon Feury MD

Please find the enclosed Mammogram and or Breast Ultrasound report for your patient:
Derrica Rucker, DOB: [REDACTED] which shows a positive finding.

Although we did fax this report to your office, by law when there is a positive finding
with Mammography and or Breast Ultrasound, we need to have a signed copy of this
letter on file confirming your receipt – acknowledgement of the diagnostic report.
Please see and sign below.

Also our facility (Jersey Diagnostic Imaging) is requesting a copy of the pathology
report on all patients whom the radiologist [REDACTED]
consultation on. Please see the enclosed signed form which the patient [REDACTED]
any pathology or consultation reports to our facility. Thank you for your cooperation
regarding this matter.

Sincerely,

Jersey Diagnostic Imaging

Dear Dr. Shannon Feury MD

Please see the following diagnostic report for your patient, which shows a positive
finding. Please sign, date, print name and fax back to our office at 908-241-0332.

Signature _____ Date _____

Printed name _____

Thank you!
JDI

2nd Request
Please sign this page
and fax back to us
9-4-2025

RUCKER, Derrica (id [REDACTED])

2025-09-04 10:28

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P 2/3



DIGITAL X-RAY • 3D MAMMOGRAPHY • ULTRASOUND • VASCULAR & ECHO ULTRASOUND • EKG

929 NORTH WOOD AVENUE • LINDEN, NJ [REDACTED]

• TEL: 908 [REDACTED]

FAX: [REDACTED]

Date: August 21, 2025

Dear Dr. Shannon Feury MD

As part of our Mammography and or Breast Ultrasound Program, an annually outcome analysis is performed on patients who had mammography reports recommending biopsies and surgical consults. This outcome analysis is part of the FDA/MQSA Quality Standards Act. We would appreciate receiving the follow-up information on the following patient:

Patient [REDACTED]

Please indicate below the appropriate follow-up information for this patient [REDACTED] this letter to our facility either by mail or fax to (908) 241-0332.

- ☐ Patient opted not to have biopsy at this time.
- ☐ Upon further evaluation, a biopsy is not indicated at this time.
- ☐ Patient [REDACTED] following:
(PLEASE ATTACH A COPY OF THE PATHOLOGY REPORT)

We thank you for your assistance in this matter. If you have any questions regarding this letter, please call (908) 241-5222.

If you have any concerns regarding the release of this information as per HIPAA regulations, Section 164.512(b) of the HIPAA regulation allows a covered entity (e.g., referring physician, pathology department, surgeon) to release patient biopsy information to a mammography facility for the purposes of the MQSA medical outcome audit without patient authorization because the disclosure: (1) is to "a person subject to FDA jurisdiction;" (2) concerns an FDA regulated product or activity for which the mammography facility has responsibility; and (3) relates to the quality, safety or effectiveness of the product or activity.

RUCKER, Derrica (id)

2025-09-04 10:29

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828 North Wood Ave. Linden NJ (P) 908-241-5222 (F) 908-241-0882
DIGITAL X-RAY • 3D MAMMOGRAPHY • ULTRASOUND • VASCULAR & ECHO ULTRASOUND • ENG

PATIENT'S NAME: RUCKER, DERRICA
DATE OF BIRTH:
DATE OF SERVICE: 20-Aug-2025 03:49:00 PM,20-Aug-2025 05:19:07 PM
PATIENT ID#: RUCDE0001
REF. DOCTOR:

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INDICATION: Left breast lump.

TECHNICAL FACTS: Bilateral CC and MLO views were performed with tomosynthesis. Spot compression views of the left breast were done. Bilateral global sonography was performed.

COMPARISON:None Available.

BREAST DENSITY: B Scattered fibroglandular tissue is seen.

FINDINGS: A triangular skin marker has been placed on the upper central left breast, denoting the site of reported palpable abnormality. There is an underlying 4 cm mass with spiculated and indistinct margins. No suspicious mammographic abnormality is present on the right.

Bilateral complete breast ultrasound was done. There is a hypochoic mass with angular margins measuring 4.1 cm at the 11 o'clock position, 10 cm from the nipple in the left breast, corresponding to the area of reported palpable abnormality. An enlarged abnormal lymph node measures 3.1 cm in the left axilla. There is also an abnormal round lymph node measuring 1 cm in the left axilla as well.

CONCLUSION: Highly suspicious left breast mass at the 11 o'clock position with suspicious corresponding axillary lymph nodes. Surgical consultation and tissue sampling are advised. The findings and recommendation were discussed with Nurse Reed over the phone on August 21, 2025 at 1:00 p.m.

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- Category 6: Biopsy Proven Malignancy

RUCKER, Derrica (id)

Lab Results

Document Information

Radiology and Imaging: Outside Radiology
MAMMO

██████████ 00:00

Attached To:

OUTSIDE RADIOLOGY ORDER ██████████

Orders Only on ██████████ with Historical Provider, ██████████

Source Information

Jaslee Quinones | Msk Pas